

CITY GENERAL HOSPITAL

Department of Pathology & Laboratory Medicine
123 Medical Drive, Mumbai - 400001 | Tel: +91-22-1234-5678

COMPLETE BLOOD COUNT (CBC) REPORT

PATIENT INFORMATION

Patient Name:	Rajesh Kumar Sharma	Patient ID:	CGH-2026-04821
Age / Gender:	45 Years / Male	Sample Date:	05-May-2026
Referring Doctor:	Dr. Priya Mehta (Cardiology)	Report Date:	06-May-2026
Clinical History:	Fatigue, breathlessness, chest pain for 2 weeks		

CBC RESULTS

Test Parameter	Result	Unit	Reference Range
Haemoglobin (Hb)	8.2 *	g/dL	13.0 - 17.0
RBC Count	3.1 *	million/uL	4.5 - 5.5
Haematocrit (PCV)	26% *	%	40 - 50%
MCV	72 *	fL	80 - 100
MCH	21 *	pg	27 - 32
MCHC	29 *	g/dL	31.5 - 34.5
RDW	18.5 *	%	11.5 - 14.5
WBC Count	11,800 *	cells/uL	4,000 - 11,000
Neutrophils	78% *	%	40 - 70%
Lymphocytes	14% *	%	20 - 40%
Monocytes	6%	%	2 - 10%
Eosinophils	2%	%	1 - 6%
Platelet Count	95,000 *	cells/uL	1,50,000 - 4,50,000
MPV	11.2	fL	7.5 - 12.5

* Values marked in RED are outside the normal reference range.

CLINICAL INTERPRETATION

- SEVERE MICROCYTIC HYPOCHROMIC ANAEMIA: Haemoglobin is critically low (8.2 g/dL). MCV (72 fL) and MCH (21 pg) are below normal, indicating iron deficiency anaemia or thalassemia trait.
- THROMBOCYTOPENIA: Platelet count is significantly reduced (95,000 cells/uL), below the lower limit of normal (1,50,000). Risk of spontaneous bleeding. Urgent clinical correlation required.
- LEUKOCYTOSIS WITH NEUTROPHILIA: WBC count is elevated (11,800) with neutrophil predominance (78%). Suggests active bacterial infection or inflammatory process.
- ELEVATED RDW (18.5%): Indicates significant anisocytosis (variation in RBC size), consistent with

mixed nutritional deficiency (iron + B12/folate).

URGENT RECOMMENDATION: Immediate haematology consultation. Consider iron studies, serum ferritin, B12, folate levels, and peripheral blood smear. Platelet count requires urgent evaluation.

Dr. Anita Desai, MD Pathology

Lab Accreditation: NABL | ISO 15189:2022

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